SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Abiraterone and prednisolone: metastatic castration-resistant prostate cancer

Metastatic castration-resistant prostate cancer · NCCP 00103 · version 9 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control prostate cancer and delay progression. The expected benefit depends on your disease setting, previous treatment and general health.

Schedule
Abiraterone 1000 mg once daily plus prednisolone 10 mg daily (or 5 mg twice daily), continuously while helping and tolerated. Continue your ADT.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Abiraterone
ANDROGEN PATHWAY TREATMENT

- ☐ High blood pressure, low potassium and fluid retention
- ☐ Liver injury and abnormal blood tests
- ☐ Heart problems, palpitations and worsening heart failure
- ☐ Tiredness, hot flushes and sexual or fertility effects alongside ADT
- ☐ Steroid effects: glucose changes, infection risk, bone loss and adrenal insufficiency if stopped inappropriately
- ☐ Muscle weakness or rare severe muscle injury; interactions with other medicines

Prednisolone
CORTICOSTEROID

- ☐ Glucose changes, increased infection risk, mood or sleep changes, stomach irritation and bone loss
- ☐ Adrenal insufficiency if stopped abruptly; altered doses may be needed during serious illness

Discuss the treatment aim, setting-specific evidence, alternatives and declining treatment; oral medicine and ADT risks, bone and cardiovascular health, sexual function, fertility, contraception, interactions, required tests and urgent contacts. Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00103 v9.

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